

June 8, 2026 12:10 AM GMT

Biopharma | North America

Takeaways from ADA Conference and KOL meeting

ADA updates from LLY, PFE, and GPCR provide further insights into the diabetes competitive landscape. We continue to believe the \$190bn market will support multiple players, with different therapies carving out roles across distinct patient populations.

LLY (OW) - exiting ADA remains extremely well positioned with one of the broadest diabetes portfolios in the industry, which we believe is a key competitive advantage: (1) Zepbound/Mounjaro (Tirzepatide; GLP-GIP) has positive head-to-head Ph3 data vs. Novo's Semaglutide (GLP) in obesity and T2D, (2) LLY has a portfolio of pipeline drugs with novel mechanisms of action such as Retatrutide (GLP-GIP-glucagon; featured at ADA) and Eloralintide (injectable selective amylin), and (3) a small molecule oral GLP-1 pill in Foundayo (notably this is not a peptide like Novo's oral Wegovy, which constrains scalability). LLY's CSO recently noted (see [HERE](#)) that Retatrutide (Reta) could represent a "workhorse" for obesity patients with lower BMI, as the low dose Ph3 data compare favorably to Tirzepatide with respect to weight loss, tolerability and time of dose escalation. We model LLY incretin franchise sales growing from ~\$60bn in 2026 to ~\$100bn in 2035. See our note from earlier this year [Tracking the GLP-1 'Unlock.'](#)

Investigators at ADA presented full data from the TRIUMPH-1/obesity and TRANSCEND-T2D-1 Ph3 trials of Reta. The presenter described this as opening a new frontier, and we view the data as establishing a new efficacy bar for incretin-based therapies, we discuss full takeaways within.

LLY guided to filing for approval of Reta for obesity in 2H26 (1H27 for T2D) and it believes Reta meets the criteria set out in the CNPV program, but declined to speculate if it could utilize this path at this point. We model Reta 2027 launch and risk-adjusted 2030 sales of \$4.6bn (70% POS). Beyond ADA, additional TRIUMPH data later this year including TRIUMPH-2 in obesity/overweight patients with T2D (including an OSA subgroup) and TRIUMPH-3 in obesity/overweight patients with established CV disease. Furthermore, we expect focus to build on TRIUMPH-5 (Reta vs. Tirzepatide) data expected in 2027 per LLY's ADA slides.

In response to a question at the event on the **Foundayo (oral GLP-1) launch**, the company noted that they are excited by what they are seeing (2 months into the launch), with great feedback and an expanding population. They indicated that they have not begun to turn on all their promotional levers. See our weekly script tracker [HERE](#).

LLY highlighted ongoing development of Eloralintide (selective for amylin receptor 1>3) and recapped the prior Ph2 data (see our thoughts on monotherapy data [HERE](#)), pointing in particular to the 3mg dose, which achieves ~12% weight loss with a placebo-like tolerability profile and does not require titration. They indicated

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North America

Industry View

In-Line

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that there are 5 Ph3 trials in progress and additional trials are planned and **they anticipate that this will become one of the largest clinical development programs in LLY's history.** They are excited about the drug and expect to report Ph2 combo data with Tirzepatide (GLP-GIP) in 2H26. We model Elora 2028 launch and risk-adjusted 2035 sales of \$5.7bn (60% POS).

There has also been growing investor interest in LLY's plans for potential longer-acting incretin(s) (i.e., monthly dosing) and at LLY's event mgmt noted that they are working on injection frequency (we would assume this would be for their existing medicines), new ultra-long acting medicines, and oral dual and triple agonists as they don't want to leave any stone un-turned in the space.

PFE (EW): Profile of Beroibenatide (Bero/PFE'3944/MET-097, injectable GLP-1) looks competitive, consistent with the top-line Ph2 release earlier this year (see [HERE](#)). At ADA there was more debate about Bero's tolerability profile rather than the efficacy profile, including the transition from weekly to monthly dosing (see within). We see the full Ph2 data as supportive of moving Bero into Ph3, and an incremental positive, but we don't expect them to meaningfully change the overall narrative on the stock at this point. We view Bero as more of a potential competitor to AMGN's Maritide (long-acting GLP-GIP), than LLY's franchise as this point. PFE is targeting a 2028 launch of Bero weekly (consistent with our model), noting that approval of monthly dosing would follow quickly thereafter. For Bero we model 2030/2035 risk adjusted sales of ~\$2bn/\$4.5bn (60% POS) or ~9% of total 2035 revs.

GPCR (OW): Presented full Aleniglipron (oral GLP-1) Ph2 data, including new data on phasing of AEs/dose interruptions, as well as interesting pre-clinical combo data for ACCG-2671 (QD oral amylin/DACRA). See our takeaways within and commentary from our recent bus tour [HERE](#).

We also hosted a meeting at the conference with an academic endocrinologist to recap/discuss the new data and implications for the future treatment paradigm in obesity and T2D.

See within for additional takeaways.



Novo is covered by Thibault Bouterin. AstraZeneca and Roche are covered by Sarita Kapila.

Endo meeting takeaways

The academic endocrinologist we hosted at ADA characterized the Reta Ph3 data (below) as impressive, with the magnitude of weight loss the best ever reported. He noted it will be an interesting question in terms of how physicians use the product. In his opinion the reduction in HbA1c for Reta is similar to Mounjaro (despite the different placebo response in the Reta trial), which suggests either a floor or that glucagon has a counter-regulatory mechanism.

Regarding Reta's safety/tolerability, he noted that the nausea rates weren't terrible, but the vomiting was high in his opinion, and lower GI side effects look comparable to other incretins. He views the urinary tract infection (UTI) imbalance as a small incremental risk, and confirmed that this is also reported with bariatric surgery, but the pathogenesis is unclear. He feels much better on the arrhythmia side given the low number of events with Reta in Ph3 (see below) and there does not appear to be a conduction defect (more atrial in nature). The number of MACE events were also small and not concerning in his view.

In his opinion the best use case for Reta is in obesity over T2D. He believes Reta could be 1L therapy in some obesity patients (particularly high BMI), but this is unlikely to move ahead of Mounjaro in T2D (here it would be used for people who tried Mounjaro that want to lose more weight). If he was a payer he would force patients to step through oral GLP-1 or Sema/Tirz before reaching Reta, and hence he does not expect utilization of Reta to be as high as Tirz. But in the DTC segment if LLY prices Reta on par with Zepbound, for example, there's no reason not to use it as 1L unless a patient is worried about potential side effects (given there's less data for Reta at this point in time).

In his view PFE's Bero (GLP) looks similar to Zepbound on efficacy in obesity, and to some degree on HbA1c lowering in T2D as well. He is interested to see what efficacy Bero can achieve as PFE doses higher and believes they need a hook if they want to be competitive in the market, but that could be the monthly dosing angle. Following recent conversations with more patients he's a believer that there will be interest/demand for monthly options, but tolerability has to be similar to existing weekly agents. In his view the tolerability profile of Bero and AMGN's Maritide (GLP-GIP) still remains a big question mark and he wants to see more data for both drugs. **But based on the existing Ph2 data, he would choose Bero over Maritide as he is more comfortable with a peptide approach over an antibody** given questions on titrating an antibody vs. a peptide, and this view is independent of any considerations of GIP antagonism. In his view vomiting is the most important side effect to compare/monitor between the drugs.

Amylins: He does not view these as front-line therapy given the data generated for the GLP-1 class on outcomes and safety, which will be hard to beat. He characterized Novo's Amycretin as incremental, and maybe better than Cagri-Sema, but was more positive on the efficacy of LLY's Elora as a monotherapy. Bigger picture, he would prefer an amylin as a stand alone therapy that he could combine with other medicines as he likes the ability to individually titrate.

Oral GLP-1s: While he is not a significant prescriber of oral GLP-1 medicines in general (they serve a limited need in his practice), he believes the profile of Foundayo is better than oral Wegovy given food and water restrictions. He has not heard any chatter on

Foundayo DDIs being an issue for patients.

Eli Lilly

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Investigators at ADA presented full data from the TRIUMPH-1 and TRANSCEND-T2D-1 Ph3 trials of Retatrutide (Reta; QW injectable GLP-1/GIP/glucagon triple agonist), following positive topline results ([LINK](#) and [LINK](#)). TRIUMPH-1 (T1) evaluates Reta vs. placebo in adults with obesity or overweight and at least one weight-related comorbidity and without diabetes, while TRANSCEND-T2D-1 (T2) is in a T2D population. The presenter described this as opening a new frontier, and we view the data as establishing a new efficacy bar for incretin-based therapies.

Reta efficacy: In **TRIUMPH-1**, Reta 12mg demonstrated weight loss of 28.3% vs. 2.2% for pbo at 80 weeks on the efficacy estimand. This compares to ~23% for Zepbound in SURMOUNT-1; see [Exhibit 15](#). In a pre-specified extension among patients with baseline BMI ≥ 35 , weight loss continued through 104 weeks, reaching 30.3% on the 12mg arm. In **TRANSCEND-T2D-1**, Reta achieved HbA1c reductions of up to ~2.0% (both estimands) and weight loss up to 16.8%/15.3% on the efficacy/treatment-regimen estimands at 40 weeks. By comparison, Mounjaro/tirzepatide in SURPASS-1 achieved A1C reductions of up to 2.07% (efficacy) and 1.75% (treatment-regimen) at 40 weeks, vs ~0% placebo, and weight loss of up to 9.5 kg (-11% at the 15 mg dose) vs 0.7 kg (-1%) for placebo on the efficacy estimand.

The ADA discussant noted that Reta's weight loss efficacy is superior to other incretins (on a cross trial basis - see Exhibits below) and has not plateaued. She also noted that it's not just about glucose lowering and weight loss, as Reta appears to have superior blood pressure reduction as well. She stressed that Reta 4mg (low dose) might be a very useful tool in weight loss clinics. The trial also included two subsets of patients with knee osteoarthritis and obstructive sleep apnea and both had clinically meaningful benefits as a result of Reta treatment. Body composition data were not collected in this trial. Lastly, the investigator for the obesity trial indicated that for the placebo arm only 75% of patients completed the study, with 64% completing treatment (vs. Reta – 89-91% completed the study, with ~75-87% completing treatment), which highlights the challenge of running a placebo controlled obesity trial, as it's hard to retain patients who are not losing weight. This theme was echoed at another Ph3 obesity presentation we attended, where it was noted that ~16% of placebo patients who dropped out of that trial independently went on treatment with a GLP-1.

Reta safety/tolerability: T1 discontinuation rates due to adverse events were 4.1%, 6.9%, and 11.3% with retatrutide 4mg, 9mg, and 12mg, respectively, vs. 4.9% with placebo. We would note discontinuation rates were generally in line with injectable obesity therapies; for context discontinuation rates due to AEs of <10% are typical for injectables. Discontinuation rates in the T2 trial were also low at 2.2%, 4.5%, and 5.1% across doses, vs. 0% for placebo, and were generally in range with those observed in Mounjaro SURPASS-1 (3–7% vs 3% placebo).

Beyond the expected GI adverse events with Reta, dysesthesia (an unpleasant, abnormal sense of touch) and urinary tract infections (UTIs) were called out as these have not generally been seen with other incretins. TRIUMPH-1 - Incidences of dysesthesia occurred in 5.1%, 12.3%, and 12.5% of patients treated with retatrutide 4mg, 9mg, and 12mg, respectively, vs. 0.9% with placebo, while incidences of urinary tract infections occurred in 7.5%, 8.8%, and 8.4% of patients treated with retatrutide 4mg, 9mg, and 12mg, respectively, vs. 5.3% with placebo, though LLY noted most dysesthesia and UTI events were mild-to-moderate and resolved during treatment. Per LLY, bariatric surgery can also lead to UTIs.

While there appeared to be a small imbalance in Reta arrhythmias in the T2D trial, there was no imbalance in the obesity trial (recall there was a signal here in Ph2).

- T2 arrhythmias and cardiac conduction disorders: Reta 4/9/12mg/pbo: 4/2/1/0
- T1 arrhythmias and cardiac conduction disorders: Reta 4/9/12mg/pbo: 1/4/5/5

The presenter for the T2D data noted that the reported arrhythmias did not increase with dose, were not serious, and no cardiac conduction disorders were recognized in the trial.

LLY previously noted that they expect data from the TRIUMPH-3 trial in the next several months to fully discharge CV risk.

There were a small number of deaths reported in both trials:

- T2 Reta 4/9/12mg/pbo: 2/0/0/0
- T1 Reta 4/9/12mg/pbo: 0/3/1/2

The two deaths in T2 (Reta 4mg arm) include a cardiac arrest in a patient who was on drug for one month, but had existing cardiac risk factors and cardiac disease, the second was a case of SCLC in a person with a history of smoking and occurred after treatment concluded. No details were provided on the deaths in T1 but the numbers are small and are generally balanced. In response to a question the presenter noted that when looking at deaths/MACE events in smaller/shorter trials there is some heterogeneity, so its more appropriate to look at these in larger outcomes trials, such as the ongoing TRIUMPH CVOT trial (data expected in 2029), which was also echoed by our KOL (above).

Exhibit 1: Reta Ph3 program summary

Trial	Setting	n	Monotherapy /Combo	Comparator	Primary Endpoint	Duration (Primary Endpoint)	Estimated Completion
TRIUMPH-1	Obesity/overweight, incl. OA and OSA subgroups	2,300	Mono	Placebo	% WL, WOMAC pain subscale score (OA), AHI (OSA)	80 weeks	Completed
TRIUMPH-2	Obesity/overweight and T2D, incl. OSA subgroup	1,000	Mono	Placebo	% WL, AHI (OSA)	80 weeks	2026
TRIUMPH-3	Obesity and CV disease	1,800	Mono	Placebo	% WL	80 weeks	2026
TRIUMPH-4	Obesity/overweight and OA	405	Mono	Placebo	WOMAC pain subscale score, % WL	68 weeks	Completed
TRIUMPH-5	Obesity	800	Mono	Tirzepatide	% WL	80 weeks	2H26
TRIUMPH-6	Obesity maintenance	643	Mono	Placebo switch	% WL	116 weeks	1H28
TRIUMPH-7	Obesity/overweight and CLBP	586	Mono	Placebo	Pain intensity per NRS, % WL	72 weeks	2H27
TRIUMPH-Outcomes	Outcomes	10,000	Mono	Placebo	Time to first occurrence of events	248 weeks	1H29
TRANSCEND-T2D-1	T2D	480	Mono	Placebo	HbA1c reduction	40 weeks	Completed
TRANSCEND-T2D-2	T2D	1,250	+ Metformin +/- SGLT2	Sema	HbA1c reduction	80 weeks	2H26
TRANSCEND-T2D-3	T2D and renal impairment	320	+ Basal Insulin +/- Metformin/SGLT2	Placebo	HbA1c reduction	52 weeks	2H26
SYNERGY-Outcomes	MASLD	4,500	Mono	Tirzepatide and placebo	Time to first occurrence of any MALO component	224 weeks*	2H30

Source: CT.gov, Morgan Stanley Research

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Beyond ADA, additional TRIUMPH data later this year including TRIUMPH-2 in obesity/overweight patients with T2D (including an OSA subgroup) and TRIUMPH-3 in obesity/overweight patients with established CV disease. Furthermore, we expect focus to build on TRIUMPH-5 (Reta vs. Tirzepatide) data expected in 2027 per LLY's ADA slides.

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Exhibit 2: Injectables obesity cross-trial efficacy (weight loss) comparison

	STEP-1	STEP-UP	SURMOUNT-1	TRIUMPH-1
N / trial duration (wks)	1961 / 68 weeks	1407 / 72 weeks	2539 / 72 weeks	2339 / 80 weeks
Comparators	Semaglutide 2.4mg Placebo	Semaglutide 2.4 mg Semaglutide 7.2mg Placebo	Tirzepatide 5mg Tirzepatide 10mg Tirzepatide 15mg Placebo	Retatrutide 4mg Retatrutide 9mg Retatrutide 12mg Placebo
Baseline Weight (kg)	105	113	104.8	112.7
Change in Weight from baseline (%) (efficacy estimand)	Placebo: -2.4 Sema 2.4mg: -16.9	Placebo: -2.4 Sema 2.4mg: -17.5 Sema 7.2mg: -20.7	Placebo: -2.4 TZP 5mg: -16.0 TZP 10mg: -21.4 TZP 15mg: -22.5	Placebo: -2.2 Reta 4mg: -19.0 Reta 9mg: -25.9 Reta 12mg: -28.3

ing JPH et al. N Engl J Med 2021;384:989-1002; Wharton S et al. Lancet Diabetes Endocrinol 2025;13:949-63; Jastreboff A et al. N Engl J Med 2022;387:205-16.

Source: ADA 2026

Exhibit 3: Injectables obesity cross-trial efficacy (weight loss thresholds) comparison

	STEP-1	STEP-UP	SURMOUNT-1	TRIUMPH-1
N / trial duration (wks)	1961 / 68 weeks	1407 / 72 weeks	2539 / 72 weeks	2339 / 80 weeks
Comparators	Semaglutide 2.4mg Placebo	Semaglutide 2.4 mg Semaglutide 7.2mg Placebo	Tirzepatide 5mg Tirzepatide 10mg Tirzepatide 15mg Placebo	Retatrutide 4mg Retatrutide 9mg Retatrutide 12mg Placebo
Baseline Weight (kg)	105	113	104.8	112.7
% achieving weight loss $\geq 5\%$	Placebo: 33.1% Sema 2.4mg: 92.4%	Placebo: 35.7% Sema 2.4mg: 92.5% Sema 7.2mg: 93.2%	Placebo: 27.9% TZP 5mg: 89.4% TZP 10mg: 96.2% TZP 15mg: 96.3%	Placebo: 34.3% Reta 4mg: 91.3% Reta 9mg: 97% Reta 12mg: 97.3%
% achieving weight loss $\geq 10\%$	Placebo: 11.8% Sema 2.4mg: 74.8%	Placebo: 20% Sema 2.4mg: 77.6% Sema 7.2mg: 86.0%	Placebo: 13.5% TZP 5mg: 73.4% TZP 10mg: 85.9% TZP 15mg: 90.1%	Placebo: 15.9% Reta 4mg: 78.4% Reta 9mg: 92% Reta 12mg: 93.2%
% achieving weight loss $\geq 15\%$	Placebo: 5% Sema 2.4mg: 54.8%	Placebo: 7.9% Sema 2.4mg: 57.5% Sema 7.2mg: 70.4%	Placebo: 6% TZP 5mg: 50.2% TZP 10mg: 73.6% TZP 15mg: 78.2%	Placebo: 7.6% Reta 4mg: 61.5% Reta 9mg: 82.8% Reta 12mg: 87.5%
% achieving weight loss $\geq 20\%$	Placebo: 2% Sema 2.4mg: 34.8%	Placebo: 2.9% Sema 2.4mg: 35.1% Sema 7.2mg: 50.9%	Placebo: 1.3% TZP 5mg: 31.6% TZP 10mg: 55.5% TZP 15mg: 62.9%	Placebo: 4.1% Reta 4mg: 44% Reta 9mg: 71.1% Reta 12mg: 76.2%
% achieving weight loss $\geq 25\%$	NR	Placebo: 0% Sema 2.4mg: 16.7% Sema 7.2mg: 33.2%	Placebo: 0.3% TZP 5mg: 16.5% TZP 10mg: 35% TZP 15mg: 39.7%	Placebo: 2.2% Reta 4mg: 27.8% Reta 9mg: 52.9% Reta 12mg: 62.5%

Source: ADA 2026

Exhibit 4: Injectables obesity cross-trial tolerability comparison

	STEP-1	STEP-UP	SURMOUNT-1	TRIUMPH-1
			2539	2339
Total Participants (N)	1961	1407	72	80
Trial duration (weeks)	68	72		
Comparators	Semaglutide 2.4 mg Placebo	Semaglutide 2.4 mg Semaglutide 7.2mg Placebo	Tirzepatide 5mg Tirzepatide 10mg Tirzepatide 15mg Placebo	Retatrutide 4mg Retatrutide 9mg Retatrutide 12mg Placebo
GI AEs (active rx vs placebo)				
Nausea	44% vs 17.4%	38.3-43.7% vs 12.9%	24.6-33% vs 9.5%	28.6-42.4% vs 14.8%
Diarrhea	31.5% vs 15.9%	27.1-27.9% vs 12.9%	18.7-23% vs 7.3%	25.2-34.1% vs 13.5%
Vomiting	24.8% vs 6.6%	16.4-24.8% vs 7%	8.3-12.2% vs 1.7%	23.8-25.3% vs 4.8%
Constipation	23.4% vs 9.5%	19.4-23.2% vs 9%	11.7-17.1% vs 5.8%	10.6-25.3% vs 10.9%
AEs of interest (active rx vs placebo)				
Dysesthesia	NR	6-22.9% vs 0.5%	NR	5.1-12.5% vs 0.9%
Gallbladder events	2.6% vs 1.2%	2.9-3% vs 0.5%	1.2-2.7% vs 1.4%	0.7-1.7% vs 0.7%
Urinary tract infections	NR	NR	NR	6.8-8.1% vs 4.8%
Injection site reaction	5% vs 6.7%	1-2.5% vs 1.5%	2.9-5.7% vs 0.3%	5.5-12.2% vs 3.8%
AEs leading to D/C (active rx vs placebo)				
Overall	7% vs 3.1%	4-5.4% vs 1%	4.3-7.1% vs 2.6%	4.1-11.3% vs 4.9%
Due to GI	4.5 vs 0.8%	2-3.3% vs 0%	1.3-2.9% vs 0.3%	2.2-4.6% vs 1.2%

Source: ADA 2026

Exhibit 5: Injectables T2D cross-trial efficacy (weight loss) comparison

	SUSTAIN-1	SUSTAIN-FORTE	STEP-UP T2D	SURPASS-1	TRANSCEND-T2D-1
N / trial duration (wks)	388 / 30 weeks	961 / 40 weeks	512 / 72 weeks	478 / 40 weeks	537 / 40 weeks
Comparators	Semaglutide 0.5mg Semaglutide 1mg Placebo	Semaglutide 1mg Semaglutide 2mg	Semaglutide 2.4 mg Semaglutide 7.2mg Placebo	Tirzepatide 5mg Tirzepatide 10mg Tirzepatide 15mg Placebo	Retatrutide 4mg Retatrutide 9mg Retatrutide 12mg Placebo
Diabetes Duration	4.2	9.5	8.4	4.7	2.5
Weight (kg)	91.9	99.3	110.1	85.9	96.9
Change in Weight from baseline (%) (efficacy estimand)	Placebo: -0.9 Sema 0.5mg: -4.0 Sema 1mg: -4.8	Sema 1mg: -6.2 Sema 2mg: -7.2	Placebo: -3.9 Sema 2.4mg: -10.7 Sema 7.2mg: -14.1	Placebo: -0.9 TZP 5mg: -7.9 TZP 10mg: -9.3 TZP 15mg: -11.0	Placebo: -2.5 Reta 4mg: -11.5 Reta 9mg: -15.5 Reta 12mg: -16.8

Sofri C et al. Lancet Diab Endocrinol 2017;251-60. Frias JP et al. Lancet Diab Endocrinol 2021;9:563-74. Lingvay J et al Lancet Diab Endocrinol 2025;13:935-48. Rosenstock J et al. Lancet 2021;398:143.

Source: ADA 2026

Exhibit 6: Injectables T2D cross-trial efficacy (HbA1c) comparison

	SUSTAIN-1	SUSTAIN-FORTE	STEP-UP T2D	SURPASS-1	TRANSCEND-T2D-1
N / trial duration (wks)	388 / 30 weeks	961 / 40 weeks	512 / 72 weeks	478 / 40 weeks	537 / 40 weeks
Comparators	Semaglutide 0.5mg Semaglutide 1mg Placebo	Semaglutide 1mg Semaglutide 2mg	Semaglutide 2.4 mg Semaglutide 7.2mg Placebo	Tirzepatide 5mg Tirzepatide 10mg Tirzepatide 15mg Placebo	Retatrutide 4mg Retatrutide 9mg Retatrutide 12mg Placebo
Diabetes Duration	4.2	9.5	8.4	4.7	2.5
Baseline HbA1c	8.05%	8.3%	8.1%	7.9%	7.93%
Change in HbA1c from baseline (efficacy estimand)	Placebo: -0.02 Sema 0.5mg: -1.5 Sema 1mg: -1.6	Sema 1mg: -1.9 Sema 2mg: -2.2	Placebo: -0.2 Sema 2.4mg: -1.6 Sema 7.2mg: -1.7	TZP 5mg: -1.9 TZP 10mg: -1.9 TZP 15mg: -2.1	Placebo: -0.8 Reta 4mg: -1.7 Reta 9mg: -2.0 Reta 12mg: -1.9
% achieving A1c < 7%	Placebo: 25% Sema 0.5mg: 74% Sema 1mg: 72%	Sema 1mg: 57.5% Sema 2mg: 67.6%	Placebo: 28.7% Sema 2.4mg: 78.1% Sema 7.2mg: 81.6%	Placebo: 19% TZP 5mg: 87% TZP 10mg: 92% TZP 15mg: 88%	Placebo: 52.1% Reta 4mg: 84% Reta 9mg: 88.9% Reta 12mg: 90.4%
% achieving A1c < 6.5%	Placebo: 13% Sema 0.5mg: 59% Sema 1mg: 60%	Sema 1mg: 38.5% Sema 2mg: 51.7%	Placebo: 16% Sema 2.4mg: 66.7% Sema 7.2mg: 72.6%	Placebo: 10% TZP 5mg: 82% TZP 10mg: 81% TZP 15mg: 86%	Placebo: 39.8% Reta 4mg: 76.7% Reta 9mg: 83.1% Reta 12mg: 85%
% achieving A1c < 5.7%	NR	NR	Placebo: 4.3% Sema 2.4mg: 17.7% Sema 7.2mg: 25%	Placebo: 1% TZP 5mg: 34% TZP 10mg: 31% TZP 15mg: 52%	Placebo: 14.1% Reta 4mg: 36.6% Reta 9mg: 45.7% Reta 12mg: 39%

Sorli C et al. Lancet Diab Endocrinol 2017;251-60. Frias JP et al. Lancet Diab Endocrinol 2021;9:563-74. Lingway I et al Lancet Diab Endocrinol 2025;13:935-48. Rosenstock J et al. Lancet 2021;398:143-55.

Source: ADA 2026

Exhibit 7: Injectables T2D cross-trial tolerability comparison

	SUSTAIN-1	SUSTAIN-FORTE	STEP-UP T2D	SURPASS-1	TRANSCEND-T2D-1
Total Participants (N)	388	961	512	478	537
Trial duration (weeks)	30	40	72	40	40
Comparators	Semaglutide 0.5mg Semaglutide 1mg Placebo	Semaglutide 1mg Semaglutide 2mg	Semaglutide 2.4 mg Semaglutide 7.2mg Placebo	Tirzepatide 5mg Tirzepatide 10mg Tirzepatide 15mg Placebo	Retatrutide 4mg Retatrutide 9mg Retatrutide 12mg Placebo
GI AEs					
Nausea	20-24% vs 8%	14-15%	28.2-29% vs 11.8%	12-18% vs 6%	16.4-26.5% vs 3.7%
Diarrhea	11-13% vs 2%	9%	18.2-20.4% vs 8.8%	12-14% vs 8%	18.7-26.3% vs 4.5%
Vomiting	4-7% vs 2%	7-8%	13.6-16.6% vs 2.9%	2-6% vs 2%	15-17.6% vs 2.2%
Constipation	4-6% vs <1%	NR	14.7-18.4% vs 4.9%	5-7% vs 1%	4.5-8.3% vs 0.7%
AEs leading to Discontinuation					
Overall	5-6% vs 2%	4-5%	5.5-5.8% vs 2.9%	3-7% vs 3%	2.2-5.1% vs 0%
Due to GI	3-4% vs <1%	3%	2.9-4.9% vs 1%	2-7% vs 1%	0-2.9% vs 0%

Sorli C et al. Lancet Diab Endocrinol 2017;251-60. Frias JP et al. Lancet Diab Endocrinol 2021;9:563-74. Lingway I et al Lancet Diab Endocrinol 2025;13:935-48. Rosenstock J et al. Lancet 2021;398:143-55.

Source: ADA 2026

Pfizer

Profile of Bero benatide (Bero/PFE'3944/MET-097, injectable GLP-1) looks competitive, consistent with the top-line Ph2 release earlier this year (see [HERE](#)). We see the full Ph2 data as supportive of moving Bero into Ph3, and an incremental positive, but we don't expect them to meaningfully change the overall narrative on the stock at this point. We view Bero as more of a potential competitor to AMGN's Maritide (long-acting GLP-GIP), than LLY's franchise as this point.

Investigators at ADA presented data from three Ph2 trials of Bero - VESPER-2 in obesity with T2D (weekly for 28 weeks), VESPER-1 open-label extension data (weekly for 28 weeks then switch to Q2W or QM through week 60 week), and VESPER-3 obesity (weekly titration, followed by monthly maintenance – interim week 28 data). Efficacy was reported as on-treatment estimand (trial product).

VESPER-2 (Bero 0.4-1.6mg QW): weight loss -2.5% to -10.2% (-0.7% pbo), HbA1c reduction -1.7 to -2.2% (-0.2% pbo), DC due to AEs 7.5% combined (0% pbo), nausea 24% combined (11.5% pbo), vomiting 18% combined (4% pbo), and diarrhea 26% combined (23% pbo).

VESPER-1 (Bero 0.4-1.2mg weekly): weight loss of -6.4% to -12.5% (+1.7% pbo) for first 28 weeks. OLE (Bero 2.4mg QM, 1.6mg Q2W, 3.2mg QM and 4.8mg QM): weight loss of -10.3% to -14.9% at week 60. DC due to AEs was 0.6% in OLE portion (patients had already received 28 weeks of weekly drug).

VESPER-3 (Bero weekly for 12 weeks, then Bero 3.2-4.8QM through week 64): weight loss of -9.8% to -12% (vs. pbo +0.2%). DC due to AEs 9.3% combined (0% pbo), nausea 38% combined (15.1% pbo), vomiting 23.3% combined (1.9% pbo), and diarrhea 14.4% combined (5.7% pbo).

For comparison we estimate LLY's Zepbound (weekly dosing) delivered placebo adjusted weight loss of ~10-14% at week 28 in SURMOUNT-1 Ph3 while AMGN's Maritide (monthly dosing) delivered weight loss of low double digit % (placebo adjusted) in Ph2.

There was more debate at ADA about Bero's tolerability profile rather than the efficacy profile. The discussant noted that in her opinion the Bero tolerability profile is the best she's seen to date for a GLP-1 drug, although she did point out that there's a spike in GI AEs when patients transition from QW to QM dosing (i.e., vomiting rates reached 24%) and she would like more insight here (we note PFE has introduced a slower step up in Ph3, below). Another well known KOL who was in the audience stated that Bero works and the data looks good, but he pushed back against the notion that the drug has the best tolerability profile and believes it is similar to other GLP-1 based medicines. One of the Bero presenters on the panel responded that the truth will come with bigger numbers in Ph3 program and hence it's best to reserve judgement until then.

Exhibit 8: GI AEs in VESPER-3

VESPER-3
Obesity/Overweight
Without T2D

Gastrointestinal Adverse Events

83% of Treated Participants Reported None or Only Mild Key GI Adverse Events^a

	Placebo, n (%) (N=53)	0.4 / 0.8 mg QW / 3.2 mg QM (N=54)	0.8 mg QW / 3.2 mg QM (N=54)	0.4 / 0.8 / 1.2 mg QW / 4.8 mg QM (N=54)	0.6 / 1.2 mg QW / 4.8 mg QM (N=53)	Berobenatide Total (N=215)
Nausea	8 (15.1)	19 (35.2)	20 (37.0)	22 (40.7)	20 (37.7)	81 (37.7)
Mild	7 (13.2)	16 (29.6)	17 (31.5)	18 (33.3)	17 (32.1)	68 (31.6)
Moderate	2 (3.8)	4 (7.4)	3 (5.6)	5 (9.3)	6 (11.3)	18 (8.4)
Severe	0	0	1 (1.9)	1 (1.9)	1 (1.9)	3 (1.4)
Vomiting	1 (1.9)	12 (22.2)	13 (24.1)	12 (22.2)	13 (24.5)	50 (23.3)
Mild	1 (1.9)	10 (18.5)	11 (20.4)	8 (14.8)	12 (22.6)	41 (19.1)
Moderate	1 (1.9)	4 (7.4)	4 (7.4)	5 (9.3)	4 (7.5)	17 (7.9)
Severe	0	0	1 (1.9)	1 (1.9)	1 (1.9)	3 (1.4)
Diarrhea	3 (5.7)	12 (22.2)	5 (9.3)	6 (11.1)	8 (15.1)	31 (14.4)
Mild	2 (3.8)	10 (18.5)	4 (7.4)	5 (9.3)	8 (15.1)	27 (12.6)
Moderate	1 (1.9)	3 (5.6)	1 (1.9)	1 (1.9)	0	5 (2.3)
Severe	0	0	0	0	0	0
Constipation	2 (3.8)	11 (20.4)	11 (20.4)	11 (20.4)	12 (22.6)	45 (20.9)
Mild	2 (3.8)	8 (14.8)	8 (14.8)	9 (16.7)	9 (17.0)	34 (15.8)
Moderate	0	3 (5.6)	3 (5.6)	2 (3.7)	3 (5.7)	11 (5.1)
Severe	0	0	0	0	0	0

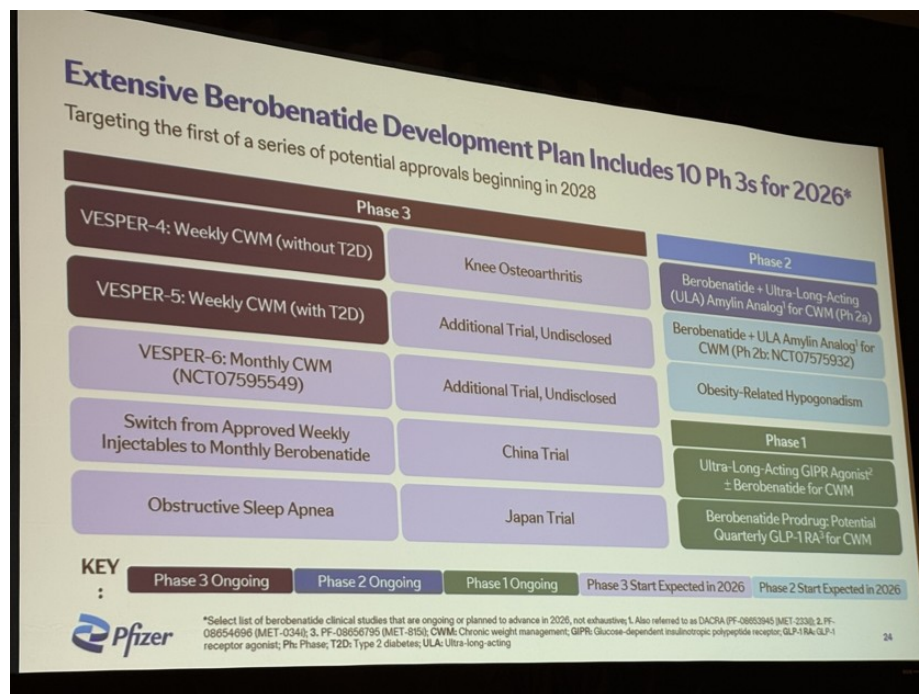
^a Nausea, vomiting, diarrhea and constipation. N=number of randomized participants. Number of unique participants for each preferred term and severity is presented. QM=monthly; QW=weekly.

Source: Company presentation

While the frequency of GI AEs with Bero was provided over time, data on the average duration of nausea/vomiting was not captured in Ph2 (but will be in Ph3). This is a question some physicians have been focused on for longer-acting monthly products, as the concern is that if the duration of side effects is longer than a weekly product, that could be a hurdle for some patients.

PFE is conducting a broad Ph3 program of Bero, with 10 trials slated to start this year (Exhibit below). The first two trials are evaluating weekly dosing in obesity (VESPER-4) and T2D (VESPER-5). They noted that these Ph3 trials use a simple dose escalation scheme that only requires 1-3 steps to reach target dose vs. ~5 steps for currently approved drugs.

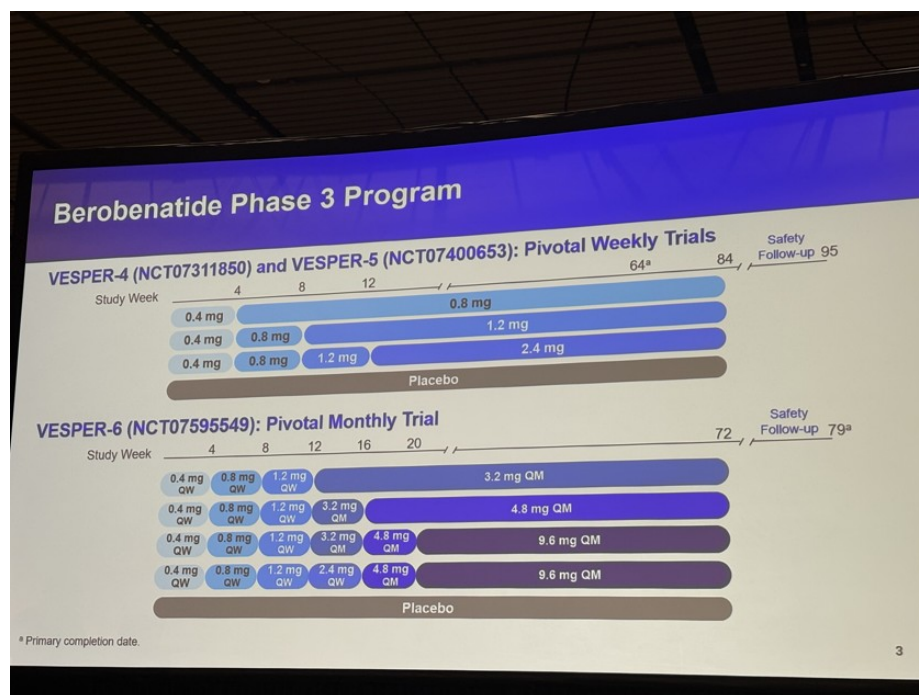
Exhibit 9: Berobenatide development program



Source: Company presentation

PFE has learned from the Bero Ph2 program (above) – they have adjusted the titration schedule, allow de-escalation of dose in the event of tolerability issues, and are also exploring a higher QM dose (9.6mg) at the top end.

Exhibit 10: Berobenatide Ph3 program



Source: Company presentation

The first Ph3 trial exploring Bero monthly dosing (VESPER-6) is now open and enrolling. The trial includes two cohorts – obesity and an obesity with T2D. The Ph3 QM

trial includes an intermediate step up of 3.2mg (the Ph2 trial escalated straight from 1.2mg to 4.8mg).

While PFE has not disclosed the full scope of the Breno Ph3 program, it will include a switch trial, but they did not commit to h2h trial(s) vs. either Semaglutide or Tirzepatide (approved weekly injectable products).

PFE is targeting a 2028 launch of Bero weekly (consistent with our model), noting that approval of monthly dosing would follow quickly thereafter. **For Bero we model 2030/2035 risk adjusted sales of ~\$2bn/\$4.5bn (60% POS) or ~9% of total 2035 revs.**

PFE indicated that it will report data from a Ph2 combo trial of Bero and PF'3945 (MET233; injectable amylin/DACRA) later this year. They have also advanced the combo into a Ph2b SOLIS-1 trial ([PCD is Aug 2027; n=872](#)). The company is also working on a Bero prodrug, which is an ultra long-acting version (half-life not disclosed), that is in Ph1.

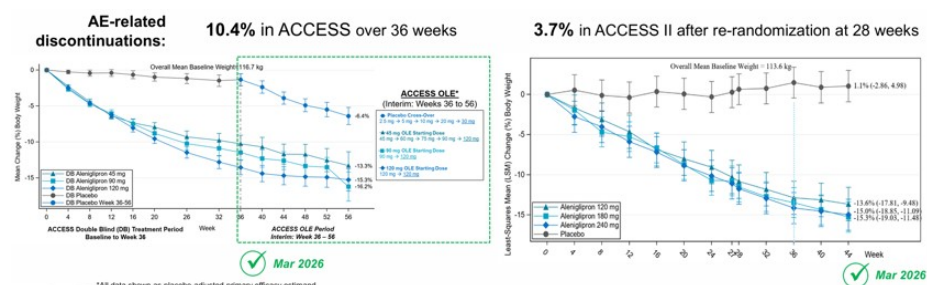
On the commercial front, PFE projected that injectables medicines (not orals) will remain the largest part of the obesity market, and have confidence that a monthly drug will unlock more new starts and also switches. They also noted that they have been a later entrant to primary care markets previously – pointing to Lipitor, Eliquis, and Prevnar as historical examples – where ultimately they were able to secure a leadership position despite arriving to market later.

Structure Therapeutics

GPCR presented full Aleniglipron (oral GLP-1) Ph2 data, including new data on phasing of AEs/dose interruptions, and pre-clinical combo data for ACCG-2671 (QD oral amylin/DACRA) in non-human primate (NHPs).

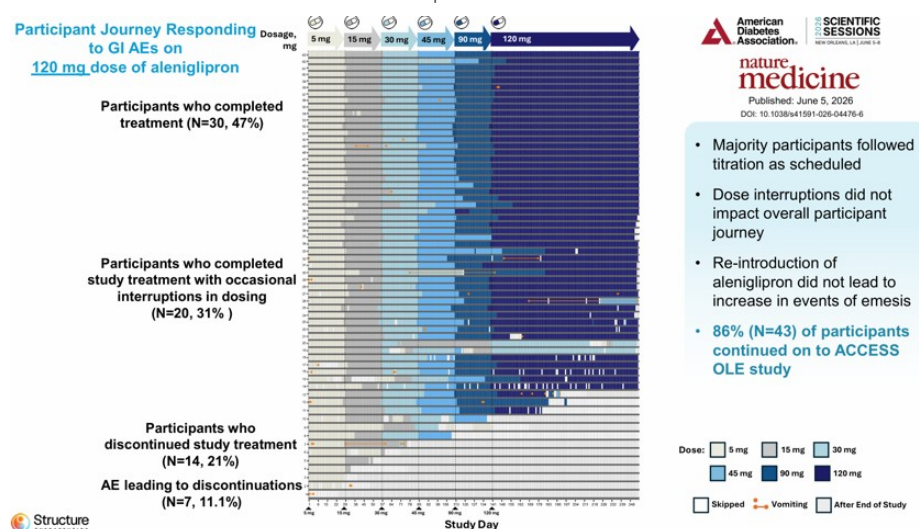
Efficacy in ACCESS Ph2 trials: Recall, GPCR previously reported top-line data from the ACCESS (45/90/120mg; [LINK](#)) and ACCESS II (incl. 180/240mg; [LINK](#)) trials, where Aleni demonstrated dose-dependent weight loss. In the ACCESS II study, Aleni achieved placebo-adjusted mean weight loss of 16.3% (39 lbs; $p<0.0001$) at the 180 mg dose and 16.0% (37 lbs; $p<0.0001$) at the 240 mg dose at 44 weeks on the efficacy estimand. Separately, in the ACCESS OLE study, Aleni achieved continued weight loss from 36 weeks, up to 16.2% (40.5 lbs) with 120 mg at 56 weeks; see summary below in [Exhibit 11](#). Both studies continue to demonstrate no evidence of weight loss plateau, which we view as encouraging. The efficacy is competitive with LLY's Orforglipron (oral GLP-1), with the typical caveats of cross trial comparisons ([Exhibit 17](#)).

Exhibit 11: Summary of Aleni ACCESS and ACCESS II efficacy data



Source: Company presentation, ADA 2026

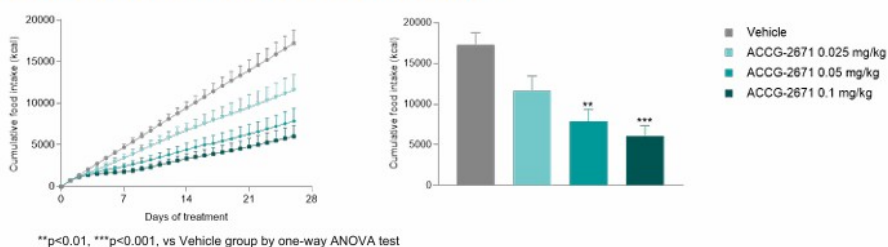
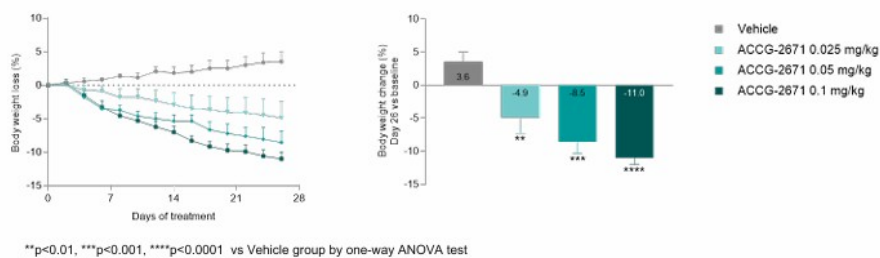
Aleni safety/tolerability: The safety and tolerability profile of Aleni is consistent with the GLP-1 class, with no apparent increase in AEs at the higher dose as reflected by the dose titration schema described below in [Exhibit 12](#). Treatment-related discontinuations were 10.4% across Aleni arms, with no events of drug-induced liver injury. Dose interruptions (indicated in the panels below as white cells) generally did not have recurrent vomiting when Aleni was re-initiated. Per the presenter, these data suggest patients can successfully restart Aleni or continue to escalate dosing following dose interruptions, which has been a focus for oral GLP-1 medicines relative to injectables.

Exhibit 12: Aleni dose escalation/interruptions

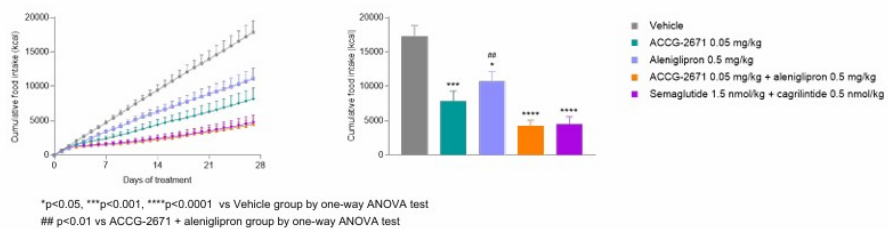
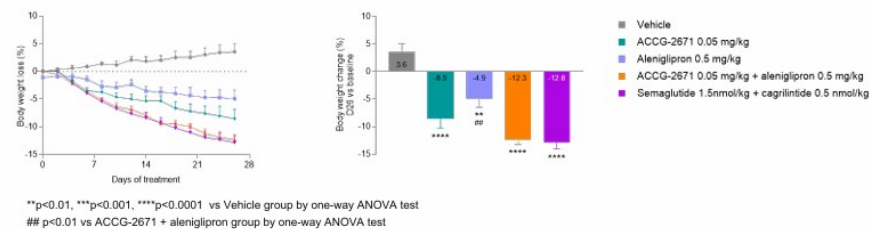
Source: Company presentation, ADA 2026

Aleni Ph3 program on track for 3Q26: The company previously received positive EOP2 feedback from the FDA and is on track to initiate its Ph3 obesity trial in 3Q26. The program will use a starting dose of 2.5mg (consistent with the body composition/OLE trials and vs. 5mg used in ACCESS/ACCESS II), given improved tolerability and immediate weight loss benefit, top dose still TBD.

ACCG-2671 (oral amylin/DACRA) pre-clinical data: GPCR presented data for ACCG-2671 as a monotherapy and in combination with aleniglipron (fixed-dose tablet) in NHPs; see poster [HERE](#). ACCG-2671 demonstrated dose-dependent responses on food intake and weight-loss as a monotherapy. The combo of ACCG-2671 (0.05 mg/kg)+Aleni (0.5mg/kg) achieved weight loss greater than that of the individual components, and appears in-line with Novo's Cagri+Sema (0.5nmol/kg Cagri + 1.5nmol/kg Sema) on both food intake and weight loss. **The Ph1 SAD trial for for ACCG-2671 monotherapy is underway, and data are expected in 3Q26.** GPCR also plans to initiate a SAD combo trial in 4Q25, likely using a different internal GLP-1 to avoid any potential impact on the perceived safety profile of Aleni as its Ph3 will be underway shortly. The company shared that ACCG-2671 has a half-life of ~60hrs, supporting QD dosing.

Exhibit 13: ACCG-2671 monotherapy NHP data**Cumulative food intake: ACCG-2671 monotreatments****Chronic body weight loss: ACCG-2671 monotreatments**

Source: Fang et al, ADA 2026

Exhibit 14: ACCG-2671+Aleni NHP data**Cumulative food intake: ACCG-2671 + aleniglipron co-administration treatment****Chronic body weight loss: ACCG-2671 + aleniglipron co-administration treatment**

Source: Fang et al, ADA 2026

Cross-trial comparisons

Exhibit 15: Injectable GLP-1 cross-trial comparison in obesity

	Obesity																
Trial	STEP-1 (Ph3)		SURMOUNT-1 (Ph3)				SURMOUNT-5 (Ph3)		Ph2				TRIUMPH-1 (Ph3)				
Company	Novo Nordisk		Eli Lilly				Eli Lilly		Eli Lilly				Eli Lilly				
Drug	Semaglutide (QW inj.)	Placebo	Tirzepatide (QW inj.)		Placebo		Tirzepatide (QW inj.)	Semaglutide (QW inj.)	Retatrutide (QW inj.)		Placebo		Retatrutide (QW inj.)		Placebo		
Class (mechanism)	GLP-1	-	GLP-1/GIP		-		GLP-1/GIP	GLP-1	GLP-1/GIP/Glucagon			-		GLP-1/GIP/Glucagon			-
Dose	2.4mg	-	5mg	10mg	15mg	-	10mg/15mg	1.7mg/2.4mg	8mg (ID 2mg)	8mg (ID 4mg)	12mg	-	4mg	9mg	12mg	-	
n	1306	655	630	636	630	643	374	376	35	35	62	70	2,339				
Background	Lifestyle Intervention		Lifestyle Intervention														
Trial Duration	68 weeks		72 weeks				72 weeks		48 weeks				80 weeks				
Titration Period	16 weeks	-	4 weeks	12 weeks	20 weeks	-	3/5 weeks	3/5 weeks	8 weeks	4 weeks	12 weeks	-	4 weeks	12 weeks	16 weeks	-	
Target Dose Tx Period	52 weeks	-	68 weeks	60 weeks	52 weeks	-	67/69 weeks	67/69 weeks	40 weeks	44 weeks	36 weeks	-	76 weeks	68 weeks	64 weeks	-	
Baseline weight, kg	105.4	105.2	102.9	105.8	105.6	104.8	112.7	113.4	106.5	108.6	108.0	109.2	112.7				
Baseline BMI	37.8	38.0	37.4	38.2	38.1	38.2	39.4	39.4	37.4	37.0	37.4	37.3	40				
% Female	73.1%	76.0%	67.6%	67.1%	67.5%	67.8%	64.7%	64.6%	48.6%	48.6%	48.4%	48.6%					
Efficacy (Treatment policy)									*								
Weight loss, absolute kg	15.3	2.6	15.4	20.6	22.1	3.2	22.8	15.5	23.5	25.9	26.2	1.8	19.8	26.7	28.2	4.4	
Weight loss, absolute %	15%	2%	15%	20%	21%	3%	20%	14%	22%	24%	24%	2%	18%	24%	25%	4%	
% Weight loss, PBO-adjusted	12%	-	12%	16%	18%	-	-	-	20%	22%	22%	-	14%	20%	21%	-	
Pts achieving ≥5% weight loss, abs %	86%	32%	85%	89%	91%	35%	82%	61%	100%	100%	100%	27%					
Pts achieving ≥5% weight loss, PBO-adj %	55%	-	51%	54%	56%	-	21%	-	73%	73%	73%	-					
Pts achieving ≥10% weight loss, abs %	69%	12%	69%	78%	84%	19%	65%	40%	90%	91%	93%	9%					
Pts achieving ≥10% weight loss, PBO-adj %	57%	-	50%	59%	65%	-	25%	-	81%	82%	84%	-					
Pts achieving ≥15% weight loss, abs %	51%	5%	48%	67%	71%	9%	48%	27%	73%	77%	83%	2%					
Pts achieving ≥15% weight loss, PBO-adj %	46%	-	39%	58%	62%	-	21%	-	71%	75%	81%	-					
Pts achieving ≥20% weight loss, abs %	32%	2%	30%	50%	57%	3%	32%	16%	50%	70%	63%	1%					
Pts achieving ≥20% weight loss, PBO-adj %	30%	-	27%	47%	54%	-	16%	-	49%	69%	62%	-					
Efficacy (Efficacy estimand)																	
Weight loss, absolute kg	17.8	2.5	16.5	22.6	23.8	2.5	24.3	17.5	23.5	25.9	26.2	1.8	21.4	29.2	31.9	2.5	
Weight loss, absolute %	17%	2%	16%	21%	23%	2%	22%	15%	22%	24%	24%	2%	19%	26%	28%	2%	
% Weight loss, PBO-adjusted	15%	-	14%	19%	20%	-	-	-	20%	22%	22%	-	17%	24%	26%	-	
Adverse events																	
GI																	
Nausea	44%	17%	25%	33%	31%	10%	44%	44%	17%	60%	45%	11%	29%	38%	42%	15%	
Vomiting	25%	7%	8%	11%	12%	2%	15%	21%	6%	26%	19%	1%	11%	23%	25%	5%	
Diarrhea	32%	16%	19%	21%	23%	7%	24%	23%	20%	20%	15%	11%	25%	34%	32%	14%	
AE-related discont. rate	7%	3%	4%	7%	6%	3%	6%	8%	14%	6%	16%	0%	4%	7%	11%	5%	

Source: Company data, Morgan Stanley Research; *all efficacy data for the Ph2 Reta trial are from an efficacy estimand

Exhibit 16: Injectable GLP-1 cross-trial comparison in T2D

Trial	SURPASS-1 (Ph3)				SUSTAIN-1 (Ph3)			REDEFINE-2 (Ph3)		TRANSCEND-T2D-1			
Company	Eli Lilly				Novo Nordisk			Novo Nordisk		Eli Lilly			
Drug	Tirzepatide (QW)			Placebo (QW)	Semaglutide (QW)		Placebo (QW)	CagriSema	Placebo	Retatrutide (QW)			Placebo
Class (mechanism)	GIP/GLP-1			-	GLP-1		-	GLP-1/amylin	-	GLP-1/GIP/Glucagon			-
Dose	5mg	10mg	15mg	-	0.5mg	1.0mg	-	2.4mg/2.4mg	-	4mg	9mg	12mg	-
n	121	121	121	115	128	130	129	1206		537			
Trial Duration	40 weeks				30 weeks			68 weeks		40 weeks			
Titration Period	4 weeks	12 weeks	20 weeks	-	4 weeks	8 weeks	-			4 weeks	12 weeks	16 weeks	-
Target Dose Tx Period	36 weeks	28 weeks	20 weeks	-	26 weeks	22 weeks	-			36 weeks	28 weeks	24 weeks	-
Efficacy (Treatment policy)													
HbA1c baseline %	7.97	7.88	7.88	8.08						7.9			
HbA1c lowering, absolute %	1.75	1.71	1.69	0.09						1.7	1.9	1.9	0.8
HbA1c lowering, PBO/CTRL-adjusted %	1.66	1.62	1.60	-						0.9	1.1	1.1	-
Pts achieving HbA1c < 7.0%, abs	87%	92%	88%	20%									
Pts achieving HbA1c < 7.0%, PBO/CTRL-adj	67%	72%	68%	-									
Pts achieving HbA1c < 5.7%, abs	34%	31%	52%	1%									
Pts achieving HbA1c < 5.7%, PBO/CTRL-adj	-66%	-69%	-48%	-									
Weight baseline, kg	87.0	85.7	85.9	84.4				102		96.9			
Weight loss, absolute kg	6.3	7.0	7.8	1.0				14.0	3.5	11.1	13.5	14.8	2.5
Weight loss, absolute (%)	7%	8%	9%	1%				13.7%	3.4%	12%	14%	15%	3%
Weight loss, PBO/CTRL-adjusted kg	6.3	7.0	7.8	-				10.5		8.6	10.9	12.3	-
Efficacy (Efficacy estimand)													
HbA1c baseline %	7.97	7.88	7.88	8.08	8.09	8.12	7.95			7.9			
HbA1c lowering, absolute %	1.87	1.89	2.07	-0.04	1.45	1.55	0.02			1.7	2.0	1.9	0.8
HbA1c lowering, PBO/CTRL-adjusted %	1.91	1.93	2.11	-	1.45	1.55	-			0.9	1.2	1.1	-
Pts achieving HbA1c < 7.0%, abs	87%	92%	88%	20%	74%	72%	25%						
Pts achieving HbA1c < 7.0%, PBO/CTRL-adj	67%	72%	68%	-	74%	72%	-						
Pts achieving HbA1c < 5.7%, abs	34%	31%	52%	1%	-	-	-						
Pts achieving HbA1c < 5.7%, PBO/CTRL-adj	-66%	-69%	-48%	-	-	-	-						
Weight baseline, kg	87.0	85.7	85.9	84.4	89.8	96.9	89.1	102		96.9			
Weight loss, absolute kg	7.0	7.8	9.5	0.7	3.73	4.53	0.98	16.0	3.2	11.1	15.0	16.3	2.4
Weight loss, absolute (%)	8%	9%	11%	1%	4%	5%	1%	15.7%	3.1%	12%	16%	17%	3%
Weight loss, PBO/CTRL-adjusted kg	7.0	7.8	9.5	-	3.7	4.5	-	12.9	-	8.7	12.6	13.9	-
Adverse events													
GI													
Nausea	12%	13%	18%	6%	20%	24%	8%			16%	20%	27%	4%
Vomiting	3%	2%	6%	2%	4%	7%	2%			16%	15%	18%	2%
Diarrhea	12%	14%	12%	8%	13%	11%	2%			19%	26%	23%	5%
AE-related discont. rate	3%	5%	7%	3%	6%	5%	2%			2%	5%	5%	0%

Source: Company data, Morgan Stanley Research

Exhibit 17: Oral GLP-1 cross-trial comparison in obesity

Trial	OASIS-1 (Ph3a)		OASIS-4		ATTAIN-1				ACCESS				ACCESS II			
Company	Novo Nordisk		Novo Nordisk		Eli Lilly				Structure Therapeutics				Structure Therapeutics			
Drug	Rybelsus (oral)	Placebo	Rybelsus	Placebo	Orforglipron (oral, QD)				Aleniglipron (oral, QD)				Aleniglipron (oral, QD)			
Class (mechanism)	GLP-1	-	GLP-1	-	GLP-1				GLP-1				GLP-1			
Dose	50mg (Obesity)	-	25mg QD	-	6mg QD	12mg QD	36mg QD	-	45mg QD	90mg QD	120mg QD	-	120mg QD	180mg QD	240mg QD	-
n	334	333	205	102	3127				45	65	64	56	8	10	9	10
Trial Duration	68 weeks		64 weeks		72 weeks				36 weeks				44 weeks			
Titration Period	16 weeks	-	12 weeks	-	8 weeks	12 weeks	16 weeks	-	12 weeks	16 weeks	20 weeks	-	20 weeks	28 weeks	32 weeks	-
Target Dose Tx Period	52 weeks	-	52 weeks	-	64 weeks	60 weeks	56 weeks	-	24 weeks	20 weeks	16 weeks	-	24 weeks	16 weeks	12 weeks	-
Efficacy (Treatment policy)																
HbA1c baseline %	5.6	5.6	5.7						5.7	5.7	5.7	5.5				
Weight baseline, kg	104.5	106.2	105.9			103.2				115.6	117.9	113.1	112.3			
Weight loss, absolute kg	15.8	2.5	14.4	2.3	7.8	8.6	11.3	2.4	10.2	12.0	12.9	1.1				
Weight loss, PBO-adjusted kg	13.2	-	12.1	-	5.4	6.2	8.9	-	9.0	10.9	11.8	-				
Weight loss, absolute (%)	15.1%	2.4%	14%	2%	8%	8%	11%	2%	9%	10%	11%	1%				
Efficacy (Efficacy estimand)																
HbA1c baseline %									5.7	5.7	5.7	5.5		5.6		5.4
Weight baseline, kg	104.5	106.2	105.9			103.2				115.6	117.9	113.1	112.3	116.2		104.3
Weight loss, absolute kg	18.2	1.9	17.6	2.9	8.0	9.4	12.4	1.0	10.4	12.6	13.7	0.9	15.8	17.8	17.4	-1.1
Weight loss, PBO-adjusted kg	16.3	-	14.7	-	7.0	8.4	11.4	-	9.5	11.7	12.8	-	17.0	18.9	18.6	-
Weight loss, absolute (%)	17.4%	1.8%	17%	3%	8%	9%	12%	1%	9%	11%	12%	1%	14%	15%	15%	-1%
Adverse events														Week 0-28		
GI																
Nausea	52%	15%	47%	19%	29%	36%	34%	10%	71%	68%	65%	21%		69%		0%
Vomiting	24%	4%	31%	6%	13%	21%	24%	4%	40%	45%	32%	5%		44%		8%
Diarrhea	27%	17%	18%	9%	21%	23%	23%	10%	42%	40%	22%	23%		36%		8%
AE-related discont. rate	6%	4%	7%	6%	5%	8%	10%	3%	13%	8%	11%	5%		28%		0%

‡ Efficacy estimand data

* Data may be from efficacy estimand

Source: Company data, Morgan Stanley Research

Valuation Methodology and Risks

Structure Therapeutics Inc (GPCR.O)

Our \$126 price target includes ~\$3.46bn in risk-adjusted revenues in 2040E (\$6.30bn unadjusted). We derive our price target from a discounted cash flow (DCF) analysis that uses a discount rate of 12.5% and a 0% terminal growth rate.

Risks to Upside

GSBR-1290 obesity data is better than anticipated and/or development timelines are accelerated.

Risks to Downside

GSBR-1290 data in obesity disappoints and/or development timelines are delayed.

Eli Lilly & Co. (LLY.N)

Our 12-month price target of \$1,344 is based on a 27x P/E multiple applied to our 2Q27-1Q28 EPS estimate of \$49.76. This multiple is in-line with LLY's 10-year average (28x) and the industry (~15x) but deserved in our view, given the company's growth profile and pipeline optionality.

Risks to Upside

- Founday/orfor launch outperforms expectations commercially
- Elora/Reta outperform expectations in clinical trials/commercially
- LLY gains share of the gvmnt obesity channel quicker than our expectations

Risks to Downside

- Founday/orfor approval is delayed or its launch underperforms commercially
- Eloralintide and Retatrutide discontinued prior to reaching the market
- Competitor data from diabetes pipeline drugs

Pfizer Inc (PFE.N)

Our 12-month price target of \$28 uses a 10x P/E multiple applied to our 2Q27-1Q28 EPS of \$2.85. This is below the peer group average of ~14-15x, but in our view is appropriate given PFE's top-line growth is declining in 2026-2030 (-5% CAGR), as well as above average patent cliff exposure vs. the peer group.

Risks to Upside

- COVID-19 testing, vaccines and treatments become mainstays
- PFE/BNTX successfully develop a COVID-19+flu combination vaccine
- PFE's LOE exposed products decline slower than expected

Risks to Downside

- COVID-19 pandemic is behind us
- PFE/BNTX's combo COVID+flu vaccine does not succeed in clinical trials
- PFE's existing franchises face commercial challenges and/or pipeline setbacks

Disclosure Section

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Therapeutics Inc, Eli Lilly & Co., Erasca, Inc., Evommune Inc, Exelixis Inc., Fractyl Health Inc, Generate Biomedicines Inc, Genmab A/S, Gilead Sciences Inc., Halozyme Therapeutics, Inc, Immix Biopharma, Immunocore Holdings Ltd, Incyte Corp, Insmed Inc, Ionis Pharmaceuticals Inc, Johnson & Johnson, Kalaris Therapeutics, Kymera Therapeutics Inc, Kyverna Therapeutics, Lakefront Biotherapeutics, Legend Biotech Corp, MapLight Therapeutics, Merck & Co., Inc., Mirum Pharmaceuticals, Moderna Inc, Monopar Therapeutics, Neurocrine Biosciences Inc, Nurix Therapeutics Inc., Organon & Co., Pfizer Inc, Pharvaris N.V., Prime Medicine Inc, ProKidney Corp, PTC Therapeutics, Recursion Pharmaceuticals Inc, Regeneron Pharmaceuticals Inc., Regenxbio Inc, Rhythm Pharmaceuticals Inc, Rocket Pharmaceuticals Inc, Royalty Pharma Plc, Sana Biotechnology, Inc, Sarepta Therapeutics Inc, Schrodinger Inc., Silence Therapeutics Plc, Structure Therapeutics Inc, Tenaya Therapeutics Inc, Trevi Therapeutics Inc, Tscan Therapeutics Inc, Ultragenyx Pharmaceutical Inc, Vertex Pharmaceuticals, Viking Therapeutics Inc, Vir Biotechnology Inc, Zenas Biopharma Inc, Zentalis Pharmaceuticals Inc.

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(as of May 31, 2026)

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Stock Rating Category	Coverage Universe		Investment Banking Clients (IBC)			Other Material Investment Services Clients (MISC)	
	Count	% of Total	Count	% of Total IBC	% of Rating Category	Count	% of Total Other MISC
Overweight/Buy	1542	42%	465	51%	30%	707	43%
Equal-weight/Hold	1571	43%	369	40%	23%	723	44%
Not-Rated/Hold	3	0%	0	0%	0%	1	0%
Underweight/Sell	551	15%	86	9%	16%	201	12%
Total	3,667		920			1632	

Data include common stock and ADRs currently assigned ratings. Investment Banking Clients are companies from whom Morgan Stanley received investment banking compensation in the last 12 months. Due to rounding off of decimals, the percentages provided in the "% of total" column may not add up to exactly 100 percent.

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Overweight (O). The stock's total return is expected to exceed the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

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Not-Rated (NR). Currently the analyst does not have adequate conviction about the stock's total return relative to the average total return of the analyst's industry (or industry team's) coverage

universe, on a risk-adjusted basis, over the next 12-18 months.

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Unless otherwise specified, the time frame for price targets included in Morgan Stanley Research is 12 to 18 months.

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Stock Price, Price Target and Rating History (See Rating Definitions)

Eli Lilly & Co. (LLY.N) - As of 06/07/26 GMT in USD
Industry : Major Pharmaceuticals



Stock Rating History: 6/1/21 : 0/A; 4/6/22 : 0/I

Price Target History: 4/28/21 : 207; 8/4/21 : 275; 1/24/22 : 272; 2/4/22 : 265; 4/6/22 : 364; 4/28/22 : 369; 7/8/22 : 395; 9/7/22 : 412; 10/12/22 : 408; 11/1/22 : 441; 11/16/22 : 436; 12/6/22 : 446; 12/13/22 : 440; 2/2/23 : 455; 3/1/23 : 444; 4/10/23 : 448; 4/13/23 : 445; 4/27/23 : 478; 5/14/23 : 507; 7/11/23 : 551; 7/21/23 : 560; 8/8/23 : 617; 9/5/23 : 640; 10/10/23 : 673; 11/2/23 : 722; 12/12/23 : 727; 1/11/24 : 763; 2/6/24 : 805; 2/16/24 : 950; 4/30/24 : 1023; 7/11/24 : 1083; 8/8/24 : 1106; 10/11/24 : 1158; 10/31/24 : 1146; 4/8/25 : 1124; 5/1/25 : 1133; 7/10/25 : 1135; 8/7/25 : 1028; 10/2/25 : 1023; 10/30/25 : 1069; 11/6/25 : 1090; 11/12/25 : 1171; 11/23/25 : 1290; 2/4/26 : 1313; 4/10/26 : 1327; 4/30/26 : 1344

Source: Morgan Stanley Research Date Format : MM/DD/YY Price Target -- No Price Target Assigned (NA)

Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —

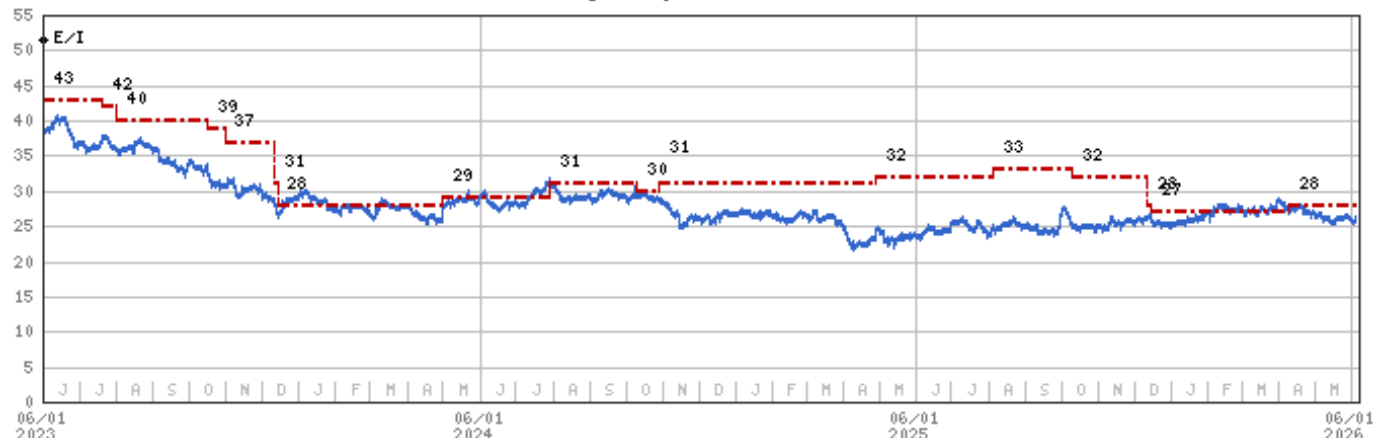
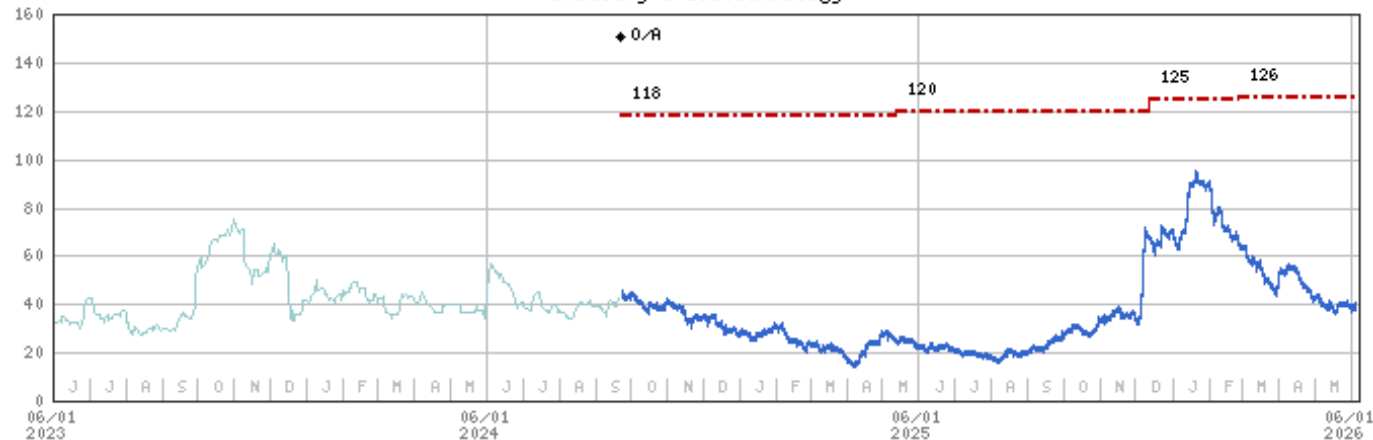
Stock and Industry Ratings (abbreviations below) appear as ♦ Stock Rating/Industry View

Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)

Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

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Pfizer Inc (PFE.N) - As of 06/07/26 GMT in USD
Industry : Major PharmaceuticalsStructure Therapeutics Inc (GPCR.O) - As of 06/07/26 GMT in USD
Industry : Biotechnology

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INDUSTRY COVERAGE: Major Pharmaceuticals

COMPANY (TICKER)	RATING (AS OF)	PRICE* (06/05/2026)
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Terence C Flynn, Ph.D.

Abbvie Inc. (ABBV.N)	O (05/11/2020)	\$227.23
Bristol Myers Squibb Co (BMY.N)	U (04/06/2022)	\$57.27
Eli Lilly & Co. (LLY.N)	O (09/03/2020)	\$1,131.42
Johnson & Johnson (JNJ.N)	O (01/28/2026)	\$232.77
Merck & Co., Inc. (MRK.N)	E (09/07/2021)	\$120.79
Organon & Co. (OGN.N)	++	\$13.36
Pfizer Inc (PFE.N)	E (07/30/2019)	\$26.04
Royalty Pharma Plc (RPRX.O)	O (05/16/2025)	\$55.87

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* Historical prices are not split adjusted.

INDUSTRY COVERAGE: Biotechnology

COMPANY (TICKER)	RATING (AS OF)	PRICE* (06/05/2026)
Judah C. Frommer, CFA		
4D Molecular Therapeutics Inc (FDMT.O)	E (11/06/2025)	\$8.64
Abivax (ABVX.O)	O (07/23/2025)	\$101.53
AgomAb Therapeutics NV (AGMB.O)	O (03/03/2026)	\$9.92
Arcutis Biotherapeutics, Inc. (ARQT.O)	O (07/02/2025)	\$21.23
Avalyn Pharma Inc (AVLN.O)	O (05/25/2026)	\$27.06
Belite Bio Inc (BLTE.O)	O (01/06/2026)	\$143.35
Bicara Therapeutics (BCAX.O)	O (10/08/2024)	\$20.00
Celldex Therapeutics Inc (CLDX.O)	O (03/20/2025)	\$28.62
Compass Pathways PLC (CMPS.O)	O (08/08/2025)	\$12.11
enGene Holdings Inc. (ENGN.O)	E (05/07/2026)	\$1.64
Evomune Inc (EVMN.N)	O (12/01/2025)	\$19.92
Genmab A/S (GMAB.O)	E (02/16/2026)	\$25.15
Immix Biopharma (IMMX.O)	O (03/25/2026)	\$7.93
Incyte Corp (INCY.O)	E (07/02/2025)	\$102.38
Kalaris Therapeutics (KLRS.O)	O (04/16/2026)	\$4.44
Kymera Therapeutics Inc (KYMR.O)	O (07/02/2025)	\$74.13
Lakefront Biotherapeutics (LKFT.O)		\$28.02
ProKidney Corp (PROK.O)	E (11/10/2022)	\$1.80
PTC Therapeutics (PTCT.O)	O (03/06/2025)	\$70.97
Regenxbio Inc (RGNX.O)	O (11/15/2024)	\$6.08
Trevi Therapeutics Inc (TRVI.O)	O (08/21/2025)	\$13.33
Zenas Biopharma Inc (ZBIO.O)	E (01/05/2026)	\$17.47
Maxwell Skor		
Adagene Inc. (ADAG.O)	E (01/30/2025)	\$3.32
Ascendis Pharma A/S (ASND.O)	O (05/05/2025)	\$210.45
Atea Pharmaceuticals Inc (AVIR.O)	E (08/12/2024)	\$4.25
Bicycle Therapeutics Plc (BCYC.O)	E (02/14/2022)	\$4.20
Crinetics Pharmaceuticals (CRNX.O)	O (01/16/2024)	\$32.92
Cytokinetics Inc (CYTK.O)	O (02/13/2025)	\$71.48
Insmid Inc (INSM.O)	O (03/30/2026)	\$94.22
Monopar Therapeutics (MNPR.O)	O (01/09/2026)	\$59.48
Pharvaris N.V. (PHVS.O)	O (08/15/2023)	\$31.09
Sana Biotechnology, Inc (SANA.O)	O (03/01/2021)	\$2.68
Tscan Therapeutics Inc (TCRX.O)	E (11/13/2025)	\$0.98
Ultragenyx Pharmaceutical Inc (RARE.O)	O (03/27/2019)	\$21.91
Michael E Ulz		
Aardvark Therapeutics Inc. (AARD.O)	U (05/15/2026)	\$3.68
Alnylam Pharmaceuticals Inc (ALNY.O)	E (09/09/2022)	\$303.05
Arrowhead Pharmaceuticals Inc (ARWR.O)	O (04/21/2026)	\$73.09

BioAge Labs Inc (BIOA.O)	E (12/05/2025)	\$15.70
Caballetta Bio Inc (CABA.O)	O (01/27/2023)	\$3.25
Dyne Therapeutics Inc (DYN.O)	O (04/30/2024)	\$17.04
Fractyl Health Inc (GUTS.O)	E (01/29/2026)	\$0.68
Ionis Pharmaceuticals Inc (IONS.O)	O (07/30/2025)	\$74.48
Kyverna Therapeutics (KYTX.O)	O (03/04/2024)	\$7.47
Mirum Pharmaceuticals (MIRM.O)	O (11/13/2023)	\$93.48
Rhythm Pharmaceuticals Inc (RYTM.O)	O (12/19/2023)	\$86.40
Rocket Pharmaceuticals Inc (RCKT.O)	E (05/27/2025)	\$2.75
Sarepta Therapeutics Inc (SRPT.O)	E (06/16/2025)	\$15.75
Silence Therapeutics Plc (SLN.O)	O (05/08/2023)	\$6.17
Tenaya Therapeutics Inc (TNYA.O)	O (08/24/2021)	\$0.73
Viking Therapeutics Inc (VKTX.O)	O (06/27/2024)	\$28.45
Vir Biotechnology Inc (VIR.O)	O (01/08/2025)	\$8.65
Zentalis Pharmaceuticals Inc (ZNTL.O)	E (06/18/2024)	\$3.66
Sean Laaman, Ph.D.		
ABSCI CORP (ABSI.O)	E (01/08/2026)	\$6.40
Acadia Pharmaceuticals Inc (ACAD.O)	E (08/06/2024)	\$21.56
Alector Inc (ALEC.O)	U (11/26/2024)	\$1.64
argenx SE (ARGX.O)	O (07/02/2025)	\$891.32
Axsome Therapeutics (AXSM.O)	E (01/08/2026)	\$232.32
BeOne Medicines (ONC.O)	O (12/02/2024)	\$270.10
Biomarin Pharmaceutical Inc (BMRN.O)	O (04/27/2026)	\$56.77
BridgeBio Oncology Therapeutics (BBOT.O)	O (12/05/2025)	\$7.81
BridgeBio Pharma Inc (BBIO.O)	O (01/06/2026)	\$67.61
Centessa Pharmaceuticals, Inc (CNTA.O)	++	\$39.62
CG Oncology (CGON.O)	O (02/19/2024)	\$53.72
Contineum Therapeutics (CTNM.O)	E (01/08/2026)	\$11.49
Cullinan Therapeutics (CGEM.O)	O (03/06/2025)	\$13.51
Denali Therapeutics Inc (DNLI.O)	O (03/06/2025)	\$19.52
Disc Medicine Inc (IRON.O)	O (11/04/2024)	\$67.00
Eikon Therapeutics Inc (EIKN.O)	O (03/02/2026)	\$8.96
Erasca, Inc. (ERAS.O)	E (08/17/2025)	\$11.97
Exelixis Inc. (EXEL.O)	E (01/08/2026)	\$52.70
Generate Biomedicines Inc (GENB.O)	O (03/24/2026)	\$12.87
Halozyne Therapeutics, Inc (HALO.O)	O (08/05/2025)	\$71.53
Immunocore Holdings Ltd (IMCR.O)	E (12/12/2024)	\$28.29
MapLight Therapeutics (MPLT.O)	O (11/21/2025)	\$28.01
Neurocrine Biosciences Inc (NBIX.O)	E (01/08/2026)	\$163.88
Recursion Pharmaceuticals Inc (RXRX.O)	E (05/22/2023)	\$3.32
Schrodinger Inc. (SDGR.O)	E (11/19/2021)	\$14.38
Terence C Flynn, Ph.D.		
Alumis Inc. (ALMS.O)	O (05/30/2025)	\$19.06
Amgen Inc. (AMGN.O)	E (10/16/2023)	\$349.58
Arcus Biosciences Inc. (RCUS.N)	E (01/08/2026)	\$23.19
Arvinas Inc (ARVN.O)	E (04/06/2022)	\$7.42
Biogen Inc (BIIB.O)	E (10/31/2024)	\$195.34
Biohaven Ltd (BHV.N)	O (07/24/2024)	\$10.80
BioNTech SE (BNTX.O)	O (09/23/2024)	\$88.08
CRISPR Therapeutics AG (CRSP.O)	U (10/11/2022)	\$51.84
Gilead Sciences Inc. (GILD.O)	O (01/10/2025)	\$129.16
Intellia Therapeutics Inc (NTLA.O)	E (01/26/2025)	\$13.54
Legend Biotech Corp (LEGN.O)	O (01/31/2022)	\$32.62
Moderna Inc (MRNA.O)	E (12/16/2020)	\$47.44
Nurix Therapeutics Inc. (NRIX.O)	O (01/08/2026)	\$14.64

Prime Medicine Inc (PRME.O)	E (11/14/2022)	\$3.06
Regeneron Pharmaceuticals Inc. (REGN.O)	E (12/03/2025)	\$635.45
Structure Therapeutics Inc (GPCR.O)	O (09/22/2024)	\$37.58
United Therapeutics Corp (UTHR.O)	E (07/11/2024)	\$549.87
Vertex Pharmaceuticals (VRTX.O)	O (12/03/2025)	\$446.83

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